Inclusive Hospital Policies, Nurse Burnout, and Job Turnover

Hyunmin Yu, José A. Bauermeister, Matthew D. McHugh, Tari Hanneman, and Karen B. Lasater

Background: Organizational turnover among nurses is associated with high levels of burnout, which may be exacerbated by workplace discrimination and exclusionary organizational policies. The theory of inclusive organizations suggests that fostering inclusive workplace environments can positively affect job retention.

Objectives: This study examined the role of inclusive hospital policies in shaping nurses' intentions to leave employment. We hypothesized that inclusive policies would reduce turnover intentions among all nurses, with a stronger effect among those experiencing high burnout.

Methods: This cross-sectional study analyzed three survey data sets from 2021: the RN4CAST-NY/IL data collected from registered nurses in New York and Illinois; the Healthcare Equality Index data, which assesses and scores hospitals' inclusion efforts for lesbian, gay, bisexual, transgender, queer or questioning, and other sexual and gender-diverse individuals; and the American Hospital Association Annual Survey. The dependent variable was nurses' intent to leave their employer within a year. The independent variable was the Healthcare Equality Index score, with nurses' high burnout serving as the moderating variable. Multilevel logistic regression with cross-level interaction was employed for the analysis.

Results: A total of 6,294 nurses from 111 hospitals were included in the study. An increase in the total Healthcare Equality Index score was associated with decreased odds of nurses intending to leave their employer. Nurses experiencing high burnout were more likely to intend to leave their employer compared to those without high burnout. Nurses' high-burnout status did not moderate the relationship between hospitals' Healthcare Equality Index scores and nurses' intent to leave.

Discussion: Inclusive hospital policies positively affect job retention regardless of nurses' burnout status. Healthcare organizations should foster inclusive and supportive environments as one strategy to improve nurse retention and organizational stability.

Key Words: inclusive hospital policies, LGBTQ+ inclusion, nurse turnover, professional burnout

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Nurse job turnover is a critical issue with broad effects on healthcare systems and patient outcomes (Hayes et al., 2012). High turnover rates increase costs for healthcare organizations due to expenses related to temporary replacement, recruitment, training, and onboarding (Hayes et al., 2012). Researchers often use *intent to leave*, defined as the likelihood of leaving one's current position within a specified timeframe as an indicator of turnover, as it is a significant predictor of actual turnover (Steel & Ovalle, 1984). Studies have identified several factors contributing to nurses' intent to leave, with burnout being a primary driver (Kelly et al., 2021; Muir et al., 2024). Burnout, characterized by emotional exhaustion, depersonalization, and diminished personal accomplishment (World Health Organization, 2019), is widespread among nurses (National Council of State Boards of Nursing, 2023) and substantially affects job satisfaction and turnover intentions (Lee et al., 2021).

Workplace discrimination and exclusion is a key organizational factor contributing to burnout among employees (Wigert & Agrawal, 2018), including nurses (Jun et al., 2023). Nurses from minoritized backgrounds, particularly racial and ethnic minorities, are disproportionately affected by burnout, partly due to workplace discrimination (Brooks Carthon et al., 2023; Brooks Carthon, Brom, et al., 2024). Racial and ethnic minority nurses report experiencing workplace racism from leaders, colleagues, and patients, as well as limited opportunities for professional advancement and a lack of safe spaces to voice these concerns and propose solutions (Brooks Carthon, Nikpour, et al., 2024).

Employees who identify as lesbian, gay, bisexual, transgender, queer or questioning, and other sexual and gender diverse (LGBTQ+) individuals also face higher levels of workplace discrimination compared to their non-LGBTQ+ counterparts (Human Rights Campaign Foundation, 2023). These discriminatory experiences include individual-level homophobic or transphobic comments, institutional-level anti-LGBTQ+ statements, and system-level exclusionary policies (Human Rights Campaign Foundation, 2023). For example, some hospitals exclude healthcare benefits for same-sex spouses and gender-affirming care in employee health plans, placing financial and emotional burdens on transgender and gender-diverse nurses (Human Rights Campaign Foundation, 2022). However, the specific experiences of discrimination and burnout among LGBTQ+ nurses have remained largely unexplored as most national nursing surveys do not systematically collect data on sexual orientation or gender identity (Health Resources & Services Administration, 2024). However, recent studies indicate that LGBTQ+ health professionals, including physicians, are more likely to report workplace discrimination and higher levels of burnout compared to non-LGBTQ+ peers (Reece-Nguyen et al., 2022; Streed et al., 2024).

To mitigate and ultimately prevent workplace discrimination, organizational LGBTQ+ inclusion efforts are crucial. The theory of inclusive organizations posits that efforts to enhance inclusion and prevent exclusion can improve organizational culture, thereby

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boosting employee well-being and retention (Shore et al., 2018). Despite the importance of inclusion in healthcare, standardized measures are limited. One key existing metric is the Healthcare Equality Index (HEI), administered biennially by the Human Rights Campaign Foundation. The HEI assesses healthcare facilities' policies and practices regarding the inclusion of LGBTQ+ patients, visitors, and employees in various areas such as nondiscrimination policies, inclusive clinical services, and employee benefits (Human Rights Campaign Foundation, 2022). Despite the theory of inclusive organizations suggesting a link between organizational inclusion and employee retention (Shore et al., 2018), the relationship between hospitals' inclusion efforts and nurses' intent to leave employment remains uncertain.

The role of nurse burnout in this relationship is also unexplored. Burnout is a strong predictor of nurse turnover (Kelly et al., 2021; Muir et al., 2024). Given that workplace discrimination is a significant organizational factor contributing to burnout, fostering an inclusive culture may have a stronger effect on nurses experiencing high levels of burnout compared to those with lower burnout levels. This suggests that burnout may moderate the relationship between a hospital's LGBTQ+ inclusion efforts and nurses' intent to leave. Specifically, nurses with higher burnout may be more sensitive to the presence (or absence) of LGBTQ+-inclusive practices in their workplace, potentially influencing their decision to stay or leave.

Addressing these gaps is crucial not only for tackling the nurse retention crisis (Lasater, 2024) but also for highlighting the potential of inclusion efforts as a strategy to mitigate turnover and provide supportive and equitable inclusive work environments for minoritized nurses. This is particularly important in the context of recent federal-level attacks on diversity, equity, and inclusion initiatives, as well as LGBTQ+ health, rights, and research (Harvard Law School Forum on Corporate Governance, 2025). This study examined the association of hospitals' LGBTQ+ inclusion efforts on nurses' intent to leave their employer and explored whether high burnout moderates this relationship (Figure 1).

Hypotheses:

1. Hospitals' LGBTQ+ inclusion efforts will be negatively associated with nurses' intent to leave.
2. High burnout among nurses will be positively associated with intentions to leave their employer.
3. The effect of hospitals' LGBTQ+ inclusion efforts on nurses' intent to leave will vary based on nurses' high-burnout status.

METHODS

Study Design and Data Source

This cross-sectional study is a secondary analysis of three survey data sets: (1) the RN4CAST-NY/IL survey, conducted from April 13 to June 22, 2021 (Aiken, Sloane, et al., 2023); (2) the HEI 2022, collected in 2021 (Human Rights Campaign Foundation, 2022); and (3) the 2021 American Hospital Association (AHA) Annual Survey (AHA, 2022). The RN4CAST-NY/IL survey was distributed to registered nurses licensed in New York (NY) and Illinois (IL) using e-mail addresses from state board nursing licensure lists. The survey captured information on nurse demographics, job outcomes, and workplace conditions from 70,072 respondents (Aiken, Sloane, et al., 2023). The AHA Annual Survey gathers detailed data on U.S. hospital operations, including staffing and services, providing insights into hospital characteristics. We restricted our analysis to 14,896 nurses who reported working in hospitals and identified their specific hospital names to ensure rigorous control for hospital-level factors that may influence nurse retention.

HEI data were linked with the RN4CAST-NY/IL data set using a common hospital identifier. From the 14,896 hospital-based nurses, we identified and included 7,343 nurses who were employed at 111 hospitals in NY and IL that participated in the HEI in 2021. The study received approval from the University of Pennsylvania Institutional Review Board. Prior to participating in the survey, all participants provided written informed consent after receiving a full explanation of the study's nature and potential consequences.

Study Variables

Dependent Variable: Intent to Leave Their Employer

Nurses' intent to leave was measured using a single-item measure (Aiken et al., 2002): "Do you plan to stay with your current employer one year from now?" The responses were binary: "yes" was coded as 0, and "no" was coded as 1, with a code of 1 indicating an intention to leave within one year.

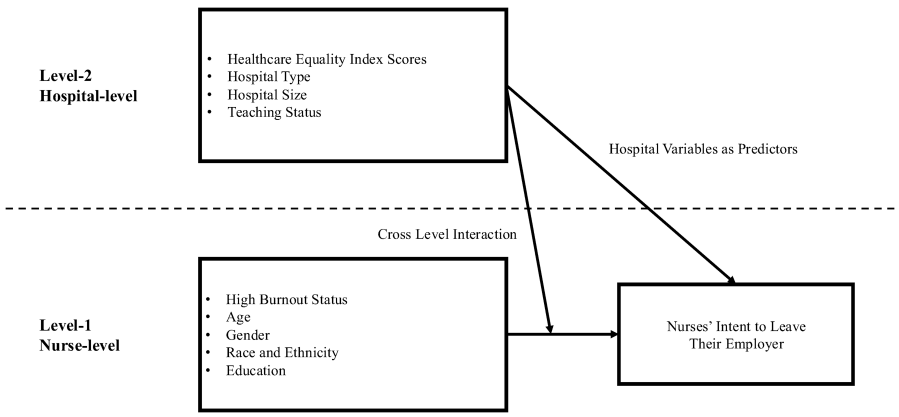

FIGURE 1. The hypothesized multilevel model. We hypothesized that LGBTQ+-inclusive hospital policies are negatively associated with nurses' intent to leave their employer, accounting for both nurse-level variables (high-burnout status, age, race and ethnicity, gender, and education) and hospital-level variables (hospital type, teaching status, and hospital size). Additionally, we hypothesized an interaction between LGBTQ+-inclusive hospital policies and high-burnout status, suggesting that the effect of LGBTQ+-inclusive hospital policies on nurses' intent to leave their employer may vary depending on whether nurses experience high burnout.

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Level 1 Independent Variable (Nurse Level): High Burnout

The Level 1 independent variable was high burnout among nurses. The nine-item Emotional Exhaustion (EE) subscale from the Maslach Burnout Inventory (Maslach & Jackson, 1981) was selected as the burnout measure because nurse burnout, as measured by this EE subscale, has been identified as a robust predictor of nurse turnover (Aiken, Lasater, et al., 2023; Kelly et al., 2021). Respondents assessed each of the nine items on a 7-point Likert scale, where answers ranged from 0 = *never* to 6 = *every day*. In our analytical sample, a Cronbach's alpha was .95. Sum scores were computed, with possible values ranging from 0 to 54. A threshold of 27 or higher was used to classify high-burnout status (Maslach et al., 1997): 0 = *no high burnout* (<27) and 1 = *high burnout* (≥27).

Level 2 Independent Variable (Hospital Level): LGBTQ+ Inclusivity in Hospital Policies

The Level 2 independent variable was the extensiveness of LGBTQ+ inclusivity within hospital policies, as measured by the HEI. The HEI utilizes a scoring scale from 0 to 100 (Human Rights Campaign Foundation, 2022). This score is derived from four distinct subcategories. *Nondiscrimination and Staff Training* assesses policies related to nondiscrimination for patients and staff as well as training programs focused on LGBTQ+ patient care (up to 40 points). *Patient Services and Support* evaluates the availability and quality of clinical services and support for LGBTQ+ individuals, including services specifically for transgender individuals (up to 30 points). *Employee Benefits and Policies* examines the benefits and policies provided to LGBTQ+ employees, such as coverage for LGBTQ+-specific healthcare (up to 20 points). *Patient and Community Engagement* assesses the institution's outreach efforts and engagement with the LGBTQ+ community through events and programs (up to 10 points). Points are deducted for recent large-scale official or public anti-LGBTQ+ incidents or the revocation of inclusive LGBTQ+ policies or practices. Further information on the HEI's scoring system and methodology is available elsewhere (Human Rights Campaign Foundation, 2020). To facilitate interpretation and improve model convergence, we rescaled the HEI score by dividing it by 10 to represent a 10-point increase rather than a 1-point increase.

Level 1 Covariates (Nurse Level)

We accounted for nurse-level (Level 1) confounding factors previously associated with nurses' intent to leave and LGBTQ+ inclusivity in hospital policies. These factors included race and ethnicity (Brooks Carthon et al., 2021; Thomas-Hawkins et al., 2022), age, gender, and educational level (Chan et al., 2013).

Age

Nurses' age was treated as a continuous variable.

Race and Ethnicity

A question about race provided the following categories: American Indian or Alaskan Native, Asian, Black or African American, Native Hawaiian or Other Pacific Islander, and White. Ethnicity was categorized as either Hispanic or non-Hispanic. We created a binary variable for race and ethnicity by combining these responses, with 0 representing non-Hispanic White and 1 denoting individuals from racial and ethnic minority groups.

Gender

A question about gender offered options for man, woman, and other, with an additional field for specification. After reviewing the written responses, we coded a category for transgender and gender-diverse individuals, including those who identify as transgender or nonbinary, alongside the cisgender categories (man and woman).

Education

A question about education was, "What is your highest level of education completed in nursing?" and had five options: hospital diploma, associate degree, baccalaureate degree, master's degree, and doctoral degree in any discipline. These were dichotomized into *hospital diploma or associate degree* and *baccalaureate degree or higher*.

Level 2 Covariates (Hospital Level)

We also accounted for hospital-level (Level 2) confounding factors previously associated with nurses' intent to leave and LGBTQ+ inclusivity in hospital policies, including hospital type (Yu et al., 2025), size, and teaching status (Aiken, Lasater, et al., 2023).

Hospital Type

Hospitals were categorized according to Definitive Healthcare's classification, which distinguishes among various patient populations and care settings (Tieche, 2024). This classification includes *children's hospitals*, which cater to patients under 18 years old; *psychiatric hospitals*, which provide continuous inpatient care for mental health conditions; *rehabilitation hospitals*, which focus on helping patients regain functional abilities; *short-term acute care hospitals*, which deliver traditional care for immediate, short-term medical issues; and *Veterans Affairs (VA) hospitals*, funded by the federal government and managed by the Veteran's Administration, which are dedicated to serving veterans and retired military members.

Teaching Status

Teaching status was determined by calculating the ratio of resident physicians and fellows to hospital beds, offering a more precise indicator of teaching intensity than university affiliations or membership in academic organizations (Ayanian et al., 1998). Hospitals were categorized as nonteaching, minor teaching (ratio of 1:4 or less), or major teaching (ratio exceeding 1:4).

Hospital Size

Hospitals were grouped based on their bed capacity into three distinct categories: small facilities with 100 or fewer beds, medium-sized hospitals accommodating between 101 and 250 beds, and large hospitals with more than 250 beds (Aiken et al., 2002).

Statistical Analysis

We conducted our analysis using Stata (Version 17, StataCorp LLC, College Station, TX) to examine the relationships between hospitals' HEI scores, nurse burnout, and nurses' intent to leave. Considering the binary outcome variable and the nested data structure, where nurses (Level 1) are nested within hospitals (Level 2), we applied multilevel logistic regression. This method effectively addresses the clustering of nurses within hospitals, enabling precise estimation of nurse- and hospital-level influences, as well as their cross-level interactions, such as the interaction between nurse- and hospital-level variables (Sommet & Morselli, 2017).

We used a two-level logistic regression model (Rabe-Hesketh & Skrondal, 2022) to investigate the effects of nurse-level (Level 1: age, gender, education, high-burnout status, race and ethnicity) and hospital-level (Level 2: HEI scores, hospital type, size, and teaching status) factors on nurses' intent to leave. In this model, the random intercept accounts for variations in nurses' intent to leave across

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hospitals, capturing differences in the probability of intent to leave between hospitals. Our analysis followed a stepwise approach, beginning with a null model that clustered by hospital and included only the binary outcome variable (nurses' intent to leave) without any independent variables. Next, we specified a Level 1 model that incorporated only nurse-level variables, followed by a Level 2 model that included both nurse- and hospital-level variables. Before fitting each model, we assessed multicollinearity by calculating generalized variance inflation factors (GVIFs). All GVIF values were low, confirming no multicollinearity concerns. To examine potential cross-level interactions, particularly whether hospitals' inclusion efforts had varying effects on nurses with different levels of burnout, we incorporated a random slope with an unstructured covariance matrix to allow for correlation between the intercept and slope (Rabe-Hesketh & Skrondal, 2022).

Last, to further assess the influence of HEI subcategories and point deductions on nurses' intent to leave, we conducted sensitivity analyses. These analyses determined whether changes in specific HEI criteria—such as nondiscrimination policies, inclusive clinical services, or employee benefits—are linked to variations in nurses' intent to leave. We also investigated the effect of point deductions within the HEI, which may reflect the presence of discriminatory practices against LGBTQ+ individuals, on the likelihood of nurses intending to leave.

Out of the 7,343 nurses in our sample, 6,294 provided complete responses for all study variables, whereas 1,049 had missing data on at least one variable (missing rate: 14.3%). Although the exact reasons for the missing data were not fully clear, our examination of missingness patterns across key variables revealed no evidence, suggesting that the missing data were dependent on either the observed data or the missing values themselves. Based on this assessment, we assumed that the data were missing completely at random. According to this assumption, using listwise deletion, which excludes observations with missing values in any study variables, should not bias the parameter estimates (Allison, 2009). Thus, we employed listwise deletion in our primary analysis, resulting in an analytic sample of 6,294 nurses. To further evaluate the robustness of our findings, we conducted a sensitivity analysis by comparing the results derived from listwise deletion with those obtained using multiple imputation, a method commonly recommended for handling missing data under less restrictive assumptions. Multiple imputation was performed using multiple imputation chain equations (White et al., 2011). Results from the multiple imputation models were consistent with those from our primary listwise deletion models, with no meaningful differences in significance levels or overall model conclusions. This consistency suggests that our approach to handling missing data did not significantly influence our findings.

RESULTS

Table 1 provides detailed hospital characteristics. Of the 111 hospitals, 84 (75.7%) were in NY, and 27 (24.3%) were in IL. The majority (82.9%) were short-term acute care hospitals. Additionally, 87 hospitals (78.3%) were teaching hospitals, and 70 (63.1%) were large hospitals with more than 250 beds. The average total HEI score was 95.6 with a standard deviation (*SD*) of 11.9. Nurse characteristics are also shown in Table 1. The average nurse age was 44.9 years (*SD* = 13.4). Of the 6,294 nurses, 3,920 (62.3%) identified as non-Hispanic White, and 5,636 (89.5%) identified as women. Additionally, 5,570 (88.5%) had a bachelor's degree or higher, and 43.3% reported high burnout.

Level 1 Model (Nurse-Level Effects)

Nurses experiencing high burnout were more likely to report an intention to leave their employer compared to those without high

TABLE 1. Hospital and Nurse Characteristics

	Total *N* = 111	
Hospital characteristics	***n***	**%**
State		
New York	84	75.7
Illinois	27	24.3
Hospital type		
Short-term acute care hospital	92	82.9
Children's hospital	3	2.7
Psychiatric hospital	3	2.7
Rehabilitation hospital	1	0.9
Veterans Affairs hospital	12	10.8
Teaching status		
None	24	21.7
Minor	38	34.2
Major	49	44.1
Hospital size		
Small (up to 100 beds)	8	7.2
Medium (101–250 beds)	33	29.7
Large (over 250 beds)	70	63.1
	M	***SD***
Total Healthcare Equality Index scores [a]	95.6	11.9
Nondiscrimination and Staff Training subscores [b]	39.3	3.5
Patient Services and Support subscores [c]	29.1	4.6
Employee Benefits and Policies subscores [d]	19.1	2.9
Patient and Community Engagement subscores [e]	8.9	2.7
Point deduction [f]	−0.7	1.8
Nurse characteristics	**Total *N* = 6,294**	
	n	**%**
Intent to leave		
Yes	1,220	19.4
No	5,074	80.6
High burnout [g]		
Yes	2,726	43.3
No	3,568	56.7
Race and ethnicity		
Non-Hispanic White	3,920	62.3
Racial and ethnic minority	2,374	37.7
Gender		
Women	5,636	89.5
Men	654	10.4
Transgender or gender diverse	4	0.1
Highest nursing education		
Hospital diploma or associate degree	724	11.5
Baccalaureate degree or higher	5,570	88.5
	M	***SD***
Age	44.9	13.4
Burnout [h]	24.4	14.4

Note. SD = standard deviation.
[a] Score ranges from 0 to 100.
[b] Subscore ranges from 0 to 40.
[c] Subscore ranges from 0 to 30.
[d] Subscore ranges from 0 to 20.
[e] Subscore ranges from 0 to 10.
[f] Point deduction ranges from −5 to 0.
[g] High burnout = a score of 27 or higher on the nine-item Emotional Exhaustion subscale of the Maslach Burnout Inventory.
[h] Score ranges from 0 to 54.

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burnout (adjusted odds ratio [aOR] = 3.85, 95% CI [3.35, 4.43]; Table 2).

Level 2 Model (Hospital-Level Effects)

Each 10-point increase in the total HEI score was associated with decreased odds of nurses intending to leave their hospital (aOR = 0.90, 95% CI [0.83, 0.98]). Compared to nurses in short-term acute care hospitals, those in VA hospitals were less likely to intend to leave their employer (aOR = 0.48, 95% CI [0.31, 0.75]). Compared to nurses in large hospitals, those in medium-sized hospitals were more likely to intend to leave their employer (aOR = 1.47, 95% CI [1.17, 1.84]). Nurses in minor teaching hospitals were less likely to intend to leave their employer compared to those in non-teaching hospitals (aOR = 0.76, 95% CI [0.58, 0.98]).

Cross-Level Interaction Effects

The interaction between hospitals' total HEI score and nurses' high-burnout status did not have a statistically significant effect ($\chi^2 = 0.02$, $p = .878$).

Sensitivity Analyses

The sensitivity analyses, which explored HEI subcategories and point deductions, revealed the following insights. Each 10-point increase in the subscore for nondiscrimination policies (aOR = 0.70, 95% CI [0.49, 0.99]), inclusive clinical services

TABLE 2. Multilevel Analysis of Predictors of Nurses' Job Turnover Intention (N = 6,294)

	Null model		Level 1 model		Level 2 model		Level 2 model with cross-level interaction	
	aOR	95% CI	aOR	95% CI	aOR	95% CI	aOR	95% CI
Intercept	0.24	***0.22, 0.27	0.14	***0.09, 0.19	0.38	*0.16, 0.89	0.41	†0.12, 1.47
Fixed effects: Nurse-level variables								
High burnout								
No (reference)								
Yes			3.85	***3.35, 4.43	3.88	***3.37, 4.46	3.54	†0.86–14.56
Age			0.99	†0.99, 1.00	0.99	†0.99, 1.00	0.99	†0.99, 1.00
Race and ethnicity								
Non-Hispanic White (reference)								
Racial and ethnic minority			1.06	†0.92, 1.22	1.08	†0.94, 1.25	1.08	†0.94, 1.25
Gender								
Women (reference)								
Men			1.13	†0.92, 1.39	1.14	†0.93, 1.40	1.14	†0.93, 1.41
Transgender or gender diverse			1.39	†0.13, 15.42	1.47	†0.13, 16.48	1.51	†0.13, 17.06
Highest nursing education								
Hospital diploma or associate degree (reference)								
Baccalaureate degree or higher			0.99	†0.80, 1.22	1.00	†0.81, 1.23	1.00	†0.81, 1.24
Fixed effects: Hospital-level variables								
Total HEI score					0.90	*0.83, 0.98	0.89	†0.79, 1.01
Hospital type								
Short-term acute care hospital (reference)								
Children's hospital					0.78	†0.47, 1.29	0.81	†0.48, 1.36
Psychiatric hospital					2.06	†0.95, 4.47	2.09	†0.95, 4.60
Rehabilitation hospital					0.56	†0.11, 2.83	0.57	†0.12, 2.85
Veterans Affairs hospital					0.48	**0.31, 0.75	0.49	**0.32, 0.77
Hospital size								
Large (over 250 beds) (reference)								
Medium (101–250 beds)					1.47	**1.17, 1.84	1.48	**1.18, 1.86
Small (up to 100 beds)					1.13	†0.71, 1.80	1.14	†0.71, 1.83
Teaching status								
None (reference)								
Minor					0.76	*0.58, 0.98	0.75	*0.58, 0.98
Major					0.94	†0.74, 1.18	0.94	†0.74, 1.18
Cross-level interaction effects								
High Burnout × HEI Score							1.01	†0.87, 1.17

Note. aOR = adjusted odds ratio; CI = confidence interval; HEI = Healthcare Equality Index.
***$p < .001$.
**$p < .01$.
*$p < .05$.
†Not significant at .05.

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(aOR = 0.82, 95% CI [0.67, 0.99]), inclusive employee benefits (aOR = 0.75, 95% CI [0.57, 0.99]), and LGBTQ+ community outreach and engagement (aOR = 0.64, 95% CI [0.44, 0.93]) was associated with decreased odds of nurses intending to leave their hospital. Conversely, point deductions for recent large-scale official or public anti-LGBTQ+ incidents or the revocation of inclusive LGBTQ+ policies or practices did not demonstrate a statistically significant effect. Additionally, the cross-level interaction between nurses' high-burnout status and any HEI subscores or point deductions did not significantly affect turnover intention.

DISCUSSION

The findings highlight the importance of inclusive policies in reducing nurses' intent to leave, supporting the theory of inclusive organizations, which posits that inclusive environments can enhance perceived inclusion and improve employee retention (Shore et al., 2018). The observed reduction in turnover intentions associated with higher HEI scores emphasizes the potential of inclusive policies to foster a more supportive work environment. This approach is crucial for retaining nursing staff, as emphasized by nurse leaders, including chief nursing officers and executives, across various U.S. health systems in a recent study, where they recommended effective strategies for LGBTQ+ inclusion in healthcare settings (Yu et al., 2024). Nurse leaders in this study underscored the need for comprehensive infrastructure support, such as LGBTQ+-inclusive electronic health records, and active engagement from both staff and leadership in LGBTQ+-inclusive initiatives, including LGBTQ+-specific training and LGBTQ+ employee resource groups. They also highlighted the importance of systematic evaluations of these efforts at the patient, employee, and organizational levels, including LGBTQ+-inclusive patient and employee experience surveys (Yu et al., 2024).

We also found that high burnout emerged as a significant predictor of nurses' intent to leave, which is consistent with our hypothesis and prior studies (Lee et al., 2021). However, even after accounting for burnout, the positive effect of LGBTQ+-inclusive hospital policies on reducing nurses' intent to leave remained significant. This persistent effect underscores the individual and organizational benefits of fostering inclusivity. Contrary to our hypothesis, nurses' high-burnout status did not moderate the relationship between LGBTQ+-inclusive hospital policies and nurses' intent to leave, as indicated by the nonsignificant cross-level interaction effect. This suggests that the benefits of inclusive policies on nurses' employment decisions may apply equally to those experiencing high levels of burnout and those who are not. This finding is particularly important as it highlights the universal value of inclusive policies across the workforce, extending beyond minoritized groups such as LGBTQ+ nurses who experience significant workplace discrimination.

Inclusive policies may positively influence non-LGBTQ+ nurses by fostering a more supportive and respectful workplace culture (Shore et al., 2018), benefiting all employees. Non-LGBTQ+ nurses may also view these efforts as reflective of broader institutional values that prioritize employee well-being and promote a positive work environment. Additionally, inclusive policies may signal to employees that the organization values belonging, fairness, and respect, not only for employees but also its patients (Brannon et al., 2018), which can boost morale and organizational commitment. Research has also shown that inclusive policies are associated with higher patient satisfaction with hospital care across all patients (Yu et al., 2025). This broad applicability is crucial for healthcare institutions seeking to improve overall retention rates, maintain a stable workforce, and enhance patient outcomes.

Our sensitivity analyses indicated that simply avoiding discrimination is not sufficient for fostering true inclusion. All measured domains—explicit nondiscrimination policies for patients and employees, inclusive clinical services that address the needs of diverse patient populations, inclusive employee benefits that support diverse staff, and LGBTQ+ community outreach demonstrating organizational commitment to community health—were significantly associated with nurses' intentions to stay with their employer. However, point deductions for organizational-level discriminatory incidents or blemishes did not show a statistically significant effect on nurses' turnover intentions. This lack of significance suggests that while eliminating discriminatory practices is essential for an inclusive workplace, the mere avoidance of discrimination does not necessarily translate into active, positive experiences that would strongly influence a nurse's decision to stay.

Limitations

A key strength of this study is its theory-based investigation of a largely underexplored area concerning the significance of organizational inclusion efforts for employee retention. Guided by the theory of inclusive organizations (Shore et al., 2018), which posits that organizational inclusion can enhance employee well-being and retention, this study tested theory-driven hypotheses about these relationships using multilevel modeling. However, several limitations must be acknowledged. The cross-sectional design limits the ability to establish causal relationships. The generalizability of the findings may also be restricted due to the focus on hospitals in only two states, NY and IL, which may limit the applicability of the results to other regions with different healthcare policies and organizational cultures. Income, which may be an important nurse-level confounder, was not included as a covariate because individual-level income data were unavailable. Another limitation is the absence of comprehensive sexual orientation and gender identity data, which prevented subgroup analyses to examine potential differential effects on LGBTQ+ nurses. Future research should prioritize collecting inclusive sex and gender measures to better understand job and health outcomes for LGBTQ+ nurses. Additionally, future studies should explore the HEI as a potential proxy for broader inclusion efforts that support diverse populations beyond LGBTQ+ individuals. Hospitals that implement policies to support one marginalized group may be more inclined to adopt inclusive practices benefiting other historically underserved communities. Investigating regional differences in the effectiveness of inclusion efforts, particularly in states with anti-LGBTQ+ laws, could provide valuable insights into protective strategies. Future research should examine how LGBTQ+ inclusion efforts affect nurses with intersecting marginalized identities to inform tailored support strategies. Lastly, the study used "intent to leave" as an outcome measure, but future research should focus on actual turnover to provide more concrete insights. Examples of such measures include overall turnover rate, which reflects the total percentage of employees who leave an organization within a given period; voluntary turnover rate, which captures employees who leave by choice; involuntary turnover rate, which measures terminations initiated by the employer; and top talent turnover rate, which tracks the departure of high-performing employees. These metrics can offer a more comprehensive understanding of workforce stability and organizational retention efforts.

CONCLUSION

Our study highlights the vital role of LGBTQ+-inclusive hospital policies in promoting nurse retention, demonstrating that such policies have a positive effect regardless of nurses' burnout status. These findings emphasize that fostering an inclusive and supportive workplace culture is not only a matter of equity but also a strategic

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approach to improving staff retention and organizational stability. By prioritizing inclusive policies, healthcare organizations can create environments where all nurses feel valued, ultimately enhancing workforce sustainability.

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